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Every time an experienced analyst leaves a health system, some of its analytics capability is lost: which tables actually mean what, which joins are safe, and why last year's metric was defined the way it was.

My paper on this was recently published in the Interactive Journal of Medical Research: "Health Care Analytics Challenges: A 3-Pillar Framework Connecting Analytics Maturity, Workforce Agility, and Technical Enablement" (doi.org/10.2196/96541).

It names the problem "institutional amnesia" and traces it to a compounding cycle: low analytics maturity, high workforce instability, and technical barriers each make the other two worse.

The proposed fix is Human-in-the-Loop Knowledge Governance (HITL-KG), which captures knowledge as durable artifacts while analysts work, rather than hoping documentation happens later. Each artifact combines a question's intent, the validated SQL that answered it, and the reasoning behind it.

The paper also includes a self-assessment rubric organizations can use to find where they're most exposed across the three pillars.



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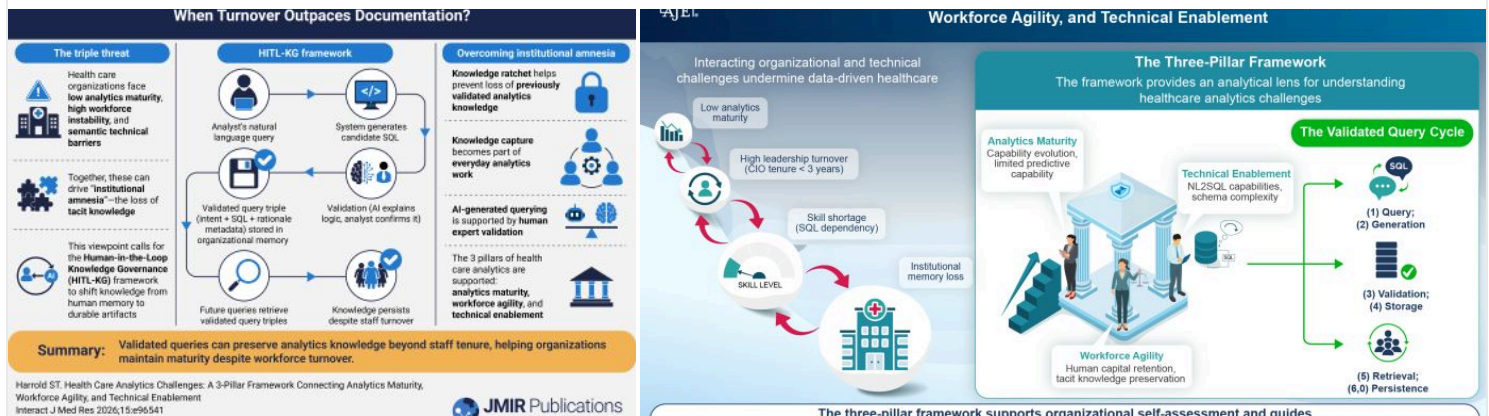
Harrold ST. Health Care Analytics Challenges: A 3-Pillar Framework Connecting Analytics Maturity, Workforce Agility, and Technical Enablement. Interact J Med Res 2026;15:e96541. doi.org/10.2196/96541

Video summary: <https://lnkd.in/epY8V4di> (Video correction: paper was published in the Interactive Journal of Medical Research, doi.org/10.2196/96541)

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If you've watched this happen at your own organization, I'd be curious how you've handled it.

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